praia health

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Submitted Via Electronic Delivery to Regulations.gov

Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS–0042–NC
P.O. Box 8013
Baltimore, MD 21244–8013

Re: Request for Information; Health Technology Ecosystem

We wish to thank the Centers for Medicare & Medicaid Services (CMS) for the opportunity to submit public comments on their Health Technology Ecosystem Request for Information. In these public comments, we focus on ways in which CMS could utilize technologies to enhance patient access, improve interoperability, and streamline identity management within healthcare systems. By addressing barriers in data availability, identity federation, and API accessibility, CMS can foster a more efficient, patient-centered digital infrastructure that reduces costs and simplifies care delivery.

Praia Health is a US-based tech firm that spun out of one of the largest nonprofit health systems to solve a pressing industry need. We help health systems give patients the same easy online and mobile tools that have become standard from banks and e-commerce industries.

Health data is locked in separate systems, forcing patients to re-establish their identity repeatedly. Providers – big and small – face high costs and technical barriers just to build basic services, while red tape raises costs and slows down access to care. Rural patients struggle the most, making even America's top healthcare services harder to access. TEFCA has improved identity authentication but lacks the required universal identity federation.

Praia Health recommends a **single, verified, federated identity** across all healthcare providers to reduce repeated logins. **Universal sign-on** should be implemented by integrating OpenID Connect (OIDC), allowing patients to log in once and access multiple providers and services. **Standardized proxy access support** should ensure caregivers can securely access patient records while defining clear rules for adding, managing, and canceling proxy relationships.

FHIR should expand beyond data sharing to make more health data actionable (i.e. enabling appointment booking, secure messaging, consent management, and care coordination) through standardized APIs. **Fair API access** must be ensured by restricting excessive fees and requiring open-access testing environments for healthcare developers.

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QUESTION	PRAIA HEALTH RESPONSE
PC-8A **PC-9A** **PC-9C**	ASTP/ONC should clarify that all data stored about a patient, including derived data, must be made programmatically available in real-time via APIs rather than locked in patient portals.
PC-9B	Documentation for API endpoints should be centrally available, and sandboxes should be rapidly and freely available to support innovation.
PC-10A	TEFCA has improved patient data access and incentivized Health IT buyers to ensure their vendors are TEFCA compliant.
PC-10C	TEFCA should expand beyond individual data access to include **all user actions,** ensuring data actionability alongside availability.
PC-10E	TEFCA lacks identity federation, preventing patients from using a single credential to access multiple portals. This must be addressed to streamline access.
PC-10G	Currently, FHIR and TEFCA are the only viable methods for digital data exchange. Paper records are impractical, and HIE access is limited.
PC-13	A clear, consumer-friendly definition of data blocking should be made available, and all systems handling patient data should include “Data Blocking Requirements” link, like California’s “Do not sell” policy, enabling direct complaint filing.
PC-14A	Health systems are restricted by their monolithic EMR vendors, as no open login requirements exist. Adopting OIDC and giving health systems the ability to select and integrate trusted digital identities.
PC-14B	Open digital identity solutions, such as IAL2-backed OIDC CIAM solution, will drive digitally engaged patients, ease care access, and reduce waste and fraud from identity confusion.
PC-14C	Digital identity solutions must be open, allowing multiple vendors to compete and provide identity services while preventing EMR vendors from restricting access. Any digital identity provider must support identity federation, including EMR vendors. Health systems must be free to add

	and select additional IDs using the OIDC standard without vendor blocking.
PC-3A **PC-3B**	Currently, non-text data is largely inaccessible via FHIR endpoints, limiting complete patient record visibility.
PC-3D	All data formats (PDF, image, free text) should be made accessible via a bulk-FHIR "/documents" endpoint to facilitate data availability and developer innovation.
PR-4	User forms of all types should be API-accessible, eliminating redundant patient inputs across platforms.
PR-5	Praia Health uses the APIs described in a, c, d, g, and h with its customers today. **Millions** already rely on these APIs to access vital care and enhance patient experiences.
PR-5H	Praia Health supports SMART on FHIR for standalone app access; however, the experience is poor, as EHR vendors add post-login screens that confuse or directly discourage patients from proceeding, even when the health system does not want these screens displayed.
PR-9A	EMR vendors limit provider capabilities and hold negotiating power over standards adoption, making certification mandates necessary to enforce open identity requirements.
PR-9B	Mandate that consumers be provided an option via an interface to disconnect approved applications, and any open identity vendor must comply with OpenID Connect 2.0.
PA-2	Tie Star-rating bonus points to payer API up-time (> 99.5 %) and full access of all data (> 95%) via open APIs.
PA-3	A direct requirement with a short transition period should be established to enforce credential portability across providers and payers, eliminating login friction, and increasing care completion rates.
TD-1	Offer an "Innovator Fast Lane" that requires access to APIs and sandboxes within 24 hours to patient portals (like EMRs). Provide grants that reward rapid innovation that measurably improves patient experience and patient outcomes.
TD-3A	Patients must have a single credential to access all their portals, apps, and tools, with identity verification easily renewed via IAL2.
TD-3B	Security is strengthened through battle-tested open standards like OpenID Connect 2.0, FIDO 2 Passkeys, and IAL2, reducing implementation risks.

TD-4	ONC certification must require vendors to offer public, no-cost, standards-based APIs for all data and actions, with proprietary solutions allowed only when parity is proven.
TD-9A	Today each implementation of an API may be slightly different, and it's impossible to fully know this unless you start to build against the API. Require API certification to standardize implementation and guarantee functionality across vendors.
TD-9B	Not all vendors are incentivized to provide open access, which may create a challenging but necessary transition.
TD-9C	API parity should be required, ensuring every health IT function has an open API. Vendors must add open API end points for new features to guarantee programmatic access.
TD-9E	Bulk data transfer and access to underlying schemas must be mandated, preventing proprietary restrictions on patient data.
TD-10	ONC Health IT certification 170.315(d)(1) should be updated to mandate OIDC certification for patient authentication and any relying party integrated applications.
TD-11	Access to this EHI data is a manual process, without programmatic access to developers or applications.
TD-11A	EHI data exports must be API-driven with defined response times.
TD-11B	Automated API-based disclosures should enable automatic response and allow for difference-since-time exports for full profile updates.
TD-11C **TD-13**	Having access to full medical records can enable patient-directed AI use cases, as well as easier data portability between points of care.
TD-18A	Restrictions on API access fees, documentation availability, and schema sharing must be eliminated. Vendors must be prevented from charging to enable FHIR APIs or using "scare screens" that discourage patients from completing SMART on FHIR workflows.

To build a patient-driven healthcare ecosystem, identity federation, open data access, and fair API policies must be mandated across all digital health platforms. Removing login barriers, standardizing patient identity, and ensuring **unrestricted API availability** will streamline care and reduce costs. Providers and payers must fully support **OIDC-based authentication**, **bulk-FHIR access**, and **transparent interoperability**. Enforcing certification standards, fair access policies, and identity portability will create a system

where patients control their data, innovation thrives, and care delivery is frictionless. We appreciate CMS' proactive efforts to leverage technology for a more efficient, patient-centered digital infrastructure.

Sincerely,

Justin Dearborn
Chief Executive Officer
Praia Health, Inc.